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Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology (ONC)
Department of Health and Human Services (HHS)
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Submitted Electronically Via [Regulations.gov](https://www.regulations.gov)

RE: [RIN 0955-AA13] Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Dear ASTP/ONC Leadership,

Thank you for the opportunity to respond to the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care. ADVION is a national organization representing health information technology (health IT) companies that develop and distribute full clinical electronic health records (EHRs), billing and point-of-care health IT systems and other software solutions serving the majority of Long Term and Post Acute Care providers (i.e., assisted living facilities (ALF), Home Health Agencies (HHAs), Inpatient Rehabilitation Facilities (IRFs), Long Term Care Hospitals (LTCHs) and Skilled Nursing Facilities (SNFs)). ADVION also represents rehabilitation therapy companies; providers of clinical laboratory and portable x-ray services; suppliers of complex medical equipment and other specialized supplies. ADVION is a founding member of the Long Term & Post-Acute Care Health IT Collaborative, which was formed in 2005 to advance health IT issues by encouraging coordination among provider organizations, policymakers, vendors, payers and other stakeholders.

The following responses are tailored to long-term post-acute care (LTPAC) and rehabilitation settings only, aligned to those questions and grounded in the RFI text.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

From a LTPAC and rehabilitation perspective, the biggest barriers cluster around infrastructure, incentives, governance, and evidence.

- Fragmented data and limited interoperability

- LTPAC and rehab providers often lack certified EHRs with modern APIs, which limits access to the longitudinal data needed to train and deploy AI safely (e.g., labs, imaging, claims, SDOH, home health notes).
 - Many facilities still rely on paper, siloed point solutions, or minimal EHR modules, so the “fuel” for AI (structured, standardized data) is missing or of poor quality.
- Misaligned payment and thin margins
 - Fee-for-service and per-diem payment in SNFs, IRFs, LTCHs, and home health create little direct financial reward for investing in AI tools that may reduce hospitalizations or improve function but do not clearly change billable units.
 - Tight operating margins in LTPAC make capital-intensive AI investments difficult absent explicit reimbursement, shared-savings participation, or clear programmatic support.
- Governance, liability, and clinical accountability gaps
 - Many LTPAC organizations lack mature AI governance structures (e.g., AI oversight committees, model risk management, bias review), creating organizational risk aversion.
 - Uncertainty about liability when AI contributes to clinical decision-making (especially without device-like FDA oversight) discourages use in nursing homes and rehab facilities that already face high litigation risk.
- Workforce, workflow, and usability challenges
 - Nursing, therapy, and medical director staffing constraints limit capacity to learn and safely integrate new AI tools into daily workflows (e.g., falls risk prediction, staffing optimization, documentation support).
 - Many tools are designed for acute care and not adapted to high-acuity, long-stay populations, leading to poor fit and low adoption in LTPAC.
- Limited LTPAC-specific evidence and benchmarks
 - Most AI evidence, benchmarks, and reference implementations focus on hospitals and ambulatory care, not skilled nursing, IRF, LTCH, or home health populations.
 - This lack of LTPAC-specific performance data makes it difficult for providers and payers to invest, particularly for smaller and rural facilities.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations. Several focused HHS actions could materially accelerate effective AI use in LTPAC and rehab without compromising safety or equity.

- Payment policy and demonstration priorities
 - Use CMMI authority to test LTPAC-inclusive models that explicitly reimburse or share savings for clinically validated AI that reduces avoidable hospitalizations, improves

functional outcomes (e.g., IRF/HH functional measures), or reduces staff burden tied to required documentation.

- Within prospective payment systems (e.g., SNF PPS under 42 CFR part 413, IRF PPS under 42 CFR part 412), clarify that reasonable AI-related clinical decision support and care management tools are allowable costs for rate-setting and recognize them in cost reports.
- Clarify AI-related expectations under existing health IT and interoperability requirements
 - Through ONC's Health IT Certification Program (45 CFR part 170), encourage or require certified health IT used in post-acute settings to support standardized data elements (e.g., USCDI-like content tailored to LTPAC assessments) and APIs that enable safe AI integration.
 - Use future ONC rulemaking and guidance to clarify expectations for AI-enabled functionality within certified EHR technology where LTPAC providers participate in ACOs or other models that depend on certified health IT.
- Privacy, security, and HIPAA guidance tailored to AI
 - Under existing HIPAA regulations (45 CFR parts 160 and 164), issue sector-specific guidance on use of AI developers and platform vendors as business associates, including model training on de-identified versus limited data sets and handling of conversational data from ambient documentation tools.
 - Highlight best practices for protecting residents' and caregivers' data when using AI tools for behavioral monitoring, falls detection, or remote monitoring in shared living environments.
- Programmatic design and technical assistance
 - Embed AI-specific technical assistance for LTPAC and rehab providers into existing HHS programs (e.g., quality improvement organizations, regional extension-type efforts) focused on selecting, validating, and safely using non-device AI tools.
 - When updating any HHS AI-related programs, explicitly include LTPAC, home health, and rehab providers as eligible participants and prioritize small, rural, and safety-net facilities.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist, and what role, if any, should HHS play to help address them? In non-device AI tools used as part of clinical care (e.g., documentation assistants, risk stratification, workflow optimization), LTPAC and rehab organizations face several distinctive legal and operational issues.

- Allocation of responsibility and liability

- When AI tools suggest care plans, staffing levels, or risk scores, it can be unclear how responsibility is shared among vendors, facility leadership, and clinicians if harm occurs, particularly where regulations emphasize interdisciplinary care planning and physician oversight.
 - HHS could develop frameworks and guidance, in partnership with other agencies where appropriate, clarifying reasonable expectations for documentation of AI use, clinician override, and institutional governance to support defensible clinical practice.
- Privacy, surveillance, and consent in congregate settings
 - Non-device AI tools that rely on video, audio, or sensor data in shared spaces (e.g., for falls or elopement detection) raise unique privacy, consent, and dignity issues for residents, family, and staff in nursing homes and rehab facilities.
 - HHS could issue best-practice guidance on consent, signage, data minimization, and secondary use of such data in LTPAC, aligned with existing HIPAA and civil rights protections, to balance safety and privacy.
- Vendor contracting and indemnification
 - Smaller LTPAC organizations often lack bargaining power to obtain balanced terms around performance guarantees, bias mitigation, incident response, and indemnification when contracting for AI tools that influence care.
 - HHS could publish model contract clauses or voluntary templates that address transparency (e.g., access to performance metrics, data provenance), audit rights, and responsibilities when AI tools materially affect resident care.
- Documentation and survey readiness
 - AI-generated notes and recommendations may not align with surveyor expectations for clear attribution, clinician signatures, or demonstration of resident-centered care planning.
 - HHS can clarify, through guidance and interpretive materials, how AI-assisted documentation can be used while maintaining compliance with existing conditions of participation and survey processes.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

LTPAC and rehab organizations need practical, low-burden evaluation methods that recognize their data and staffing constraints.

- Pre-deployment and post-deployment methods

- Prospective “silent mode” evaluations in which AI tools run in the background without affecting care, comparing AI outputs with existing decisions and resident outcomes, can be feasible in SNFs, IRFs, and home health, especially for readmission, falls, or pressure injury risk prediction.
 - Ongoing post-deployment monitoring using simple dashboards (e.g., calibration drift, differential performance by race, language, cognitive status, or payer) can fit within existing quality programs if supported by vendors.
- Human-centered and workflow evaluations
 - Usability testing with nursing, therapy, and aide staff in real LTPAC workflows (med passes, therapy scheduling, restorative nursing) is critical, given high turnover and varied digital literacy.
 - Simulation-based assessments (e.g., mock care conferences, documentation exercises) can reveal where AI tools add burden or risk, particularly in interdisciplinary care planning.
- HHS support mechanisms
 - Grants and cooperative agreements could fund shared LTPAC “testbeds” where vendors and providers evaluate AI tools using standardized measures and common data sets.
 - Prize competitions could target AI tools that demonstrate superior performance and usability in post-acute outcomes (e.g., functional gains, community discharge, rehospitalization), with clearly defined, transparent evaluation protocols.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS can play an enabling role by recognizing and aligning with private sector initiatives that set quality bars for AI in clinical care.

- Signal and alignment from HHS programs
 - HHS could recognize independent AI accreditation or certification bodies that meet transparent criteria (e.g., documented bias testing, post-deployment monitoring, user-centered design) and allow participation or scoring advantages in relevant HHS programs when accredited tools are used in LTPAC settings.
 - Quality reporting and value-based programs that include LTPAC and rehab providers could incorporate optional measures or attestations related to use of accredited AI tools that support quality and safety.
- Support for sector-specific standards
 - HHS could convene LTPAC-focused stakeholders to define baseline expectations for AI tools used in nursing homes, IRFs, LTCHs, and home health (e.g., data elements, minimal performance metrics, explainability), which private accreditors could adopt.

- Technical assistance and shared resources (e.g., checklists) can help smaller providers understand and request accredited/certified AI solutions during procurement.
- Workforce credentialing and training
 - HHS could support curricula or micro-credentials for LTPAC medical directors, DONs, therapists, and quality leaders on safe AI use, bias awareness, and governance, which in turn supports safer adoption of accredited tools.
 - Such training could be integrated into existing quality improvement and leadership development programs that HHS or its contractors already support for LTPAC settings.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

In LTPAC and rehabilitation, many AI tools are emerging, but performance and cost experience is mixed.

- Areas where AI has met or exceeded expectations
 - Ambient clinical documentation and coding support can reduce time spent on notes and MDS/IRF-PAI/HH documentation, decreasing burden for clinicians and enabling more face-to-face care when well-implemented.
 - Predictive models for avoidable hospitalizations, when integrated into interdisciplinary care management and nurse triage workflows, can help prioritize at-risk residents and support timely interventions.
- Areas where AI has fallen short
 - Tools trained primarily on hospital or outpatient data often perform poorly on frail, cognitively impaired, and multi-morbid LTPAC populations, leading to low trust and limited clinical uptake.
 - Some solutions increase documentation or alert fatigue, adding complexity to already stretched workflows and eroding any projected ROI.
- High-potential, novel tools for LTPAC and rehab
 - AI-enabled functional status and mobility prediction to tailor therapy intensity, anticipate discharge needs, and reduce rehospitalization risk in IRFs, SNFs, LTCHs, and home health.
 - AI tools that support behavioral health and dementia care in LTPAC (e.g., early detection of behavioral escalation, personalized non-pharmacologic interventions) while respecting residents' rights and privacy.
 - Workforce optimization tools that align staffing, skill mix, and scheduling with dynamic resident acuity and fall/pressure injury risk, supporting both quality and cost containment.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Adoption of AI in LTPAC and rehabilitation is shaped by a distinct set of roles and administrative constraints.

- Influential decision-makers in LTPAC and rehab
 - Facility and system leadership (CEOs, administrators, CFOs) and clinical leaders (directors of nursing, medical directors, therapy directors) are primary decision-makers for AI that affects care delivery and staffing.
 - At multi-facility organizations, corporate compliance, IT, and quality leaders often act as gatekeepers, assessing legal risk, interoperability, and alignment with quality and survey requirements.
- Primary administrative hurdles
 - Lengthy contracting, security, and legal review processes-compounded by limited internal expertise on AI risk-delay or prevent implementation, especially in smaller organizations.
 - Survey, compliance, and documentation concerns create additional hurdles, as leaders worry that AI-generated outputs could conflict with regulatory expectations or be difficult to explain to surveyors and auditors.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability is foundational to effective AI in LTPAC and rehabilitation.

- Data types and standards
 - Consistent, computable access to post-acute assessment data (e.g., MDS items, IRF-PAI, LTCH CARE data set, OASIS), medications, vital signs, problem lists, functional scores, and social needs is critical for accurate AI.
 - Continued evolution and use of standard vocabularies and data models relevant to LTPAC (e.g., alignment of assessment items with standardized codes and FHIR resources) would support safer model development and deployment.
- Interfaces and benchmarking tools

- APIs that enable bidirectional exchange between hospital EHRs, LTPAC EHRs, and AI tools (e.g., for transition-of-care summaries, risk scores, and care plans) would expand markets and facilitate multi-site evaluations.
- Shared, de-identified benchmarking data sets spanning acute, post-acute, and home-based care could allow developers to test models across care settings, including LTPAC and rehab, before deployment.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers in LTPAC and rehabilitation settings face distinctive challenges that AI could help address, alongside notable concerns.

- Challenges AI could help address
 - Need for more timely recognition of clinical deterioration, pain, depression, and functional decline in residents who cannot easily advocate for themselves, including those with cognitive impairment.
 - Desire for better communication about care goals, progress in rehabilitation, and discharge planning across multiple providers and family caregivers.
- Key concerns about AI use
 - Fear that AI will replace human interaction and reduce the personal, relational aspects of care that are central to long-stay environments like nursing homes.
 - Worries about surveillance, loss of privacy, and data misuse when sensors, cameras, or conversational systems are introduced into resident rooms and common spaces.
 - Concern that AI might introduce or amplify bias, resulting in lower quality care or fewer rehabilitative opportunities for certain groups (e.g., by age, disability, race, language, or payer).

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

- a. Are there published findings about the impact of adopted AI tools and their use clinical care?
- b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

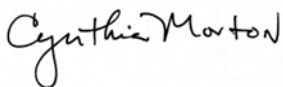
LTPAC and rehabilitation would benefit from targeted AI research and translation efforts.

- Priority research areas for HHS

- Development and validation of AI models specifically for LTPAC and rehab populations (e.g., frail older adults, individuals with complex disabilities) using representative data from SNFs, IRFs, LTCHs, and home health.
 - Research on AI tools that support functional recovery, safe transitions, medication management, and dementia care in long-term and post-acute settings, including equity and bias assessments.
- Published findings on AI impact in clinical care (general)
 - Existing literature described in the RFI underscores that AI has shown promise in clinical prediction, workflow support, and imaging analysis, while highlighting ongoing concerns related to bias, generalizability, and real-world performance.
 - HHS's interest in aligning regulation, reimbursement, and research & development reflects recognition that translating such findings into safe, scalable AI use in everyday care-including LTPAC and rehabilitation-requires coordinated policy attention.
- Approaches to costs, benefits, and transfers
 - The RFI notes that HHS aims to ensure AI is deployed in ways that enhance productivity, reduce burden, lower health care costs, and improve outcomes, indicating a focus on total system value rather than isolated procedure-level savings.
 - For LTPAC and rehab, this implies research on how AI affects rehospitalizations, length of stay, staffing efficiency, and caregiver burden, as well as distributional effects across payers, providers, and patients.

Thank you again for allowing us to provide responses on this RFI. I am available to provide additional input or clarification and can be reached at 202 803-2385 or Cynthia@ADVIONadvocates.org.

Sincerely,



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CEO